PEDIATRIC MEDICINE · LECTURE 2 · QUESTION BANK

Congenital Heart Disease

عيوب القلب الخلقية — بنك الأسئلة

106	6	12
Past exam questions	NCLEX-style	Flashcards

المصادر اللي اتجمعت منها أسئلة المحاضرة دي:

ميدترم 2021 (أطفال نظري) · كويز 2 – 2022 (أطفال نظري) · كويز 2 – 2024 (أطفال نظري) · كويز 2 شعبة 1 (أطفال نظري) · كويز 2 شعبة 3 (أطفال نظري) · كويز 2 أطفال نظري · أسئلة سابقة

جنب كل سؤال مصدره. لو السؤال اتكرر في كذا امتحان مكتوب كل مرة بمصدرها، وعليه علامة "تكرر". الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1 ميدترم 2021 (أطفال نظري)

An appropriate nursing action related to the administration of digoxin (Lanoxin) to an infant would be:

a. Monitor serum calcium
b. Checking respiratory rate
c. Repeating a dose if the child vomits
d. Give dose if heart rate is 100 beats/min

2 ميدترم 2021 (أطفال نظري)

The nurse can interpret that a ventricular septal defect does not cause cyanosis because of:

a. Blood shunt from left to right
b. Increased pressure in the right atrium
c. Decreased pressure in the left atrium
d. Blood shunt from right to left

3 ميدترم 2021 (أطفال نظري)

Patent ductus arteriosus is an opening between the pulmonary artery and aorta. (True / False)

a. True
b. False

4 كويز 2 – 2022 (أطفال نظري)

The most important nursing intervention for a child with congenital heart disease on digoxin is: تكرر في 2 مصادر

a. Assess heart rate
b. Assess respiratory rate
c. Assess K level in blood
d. Assess intake and output

5 Which of the following should the nurse expect to note as a frequent complication for a child with congenital heart disease? تكرر في 3 مصادر

كويز 2 – 2022 (أطفال نظري)

a. Seizure disorder
b. Bleeding tendencies
c. Frequent vomiting and diarrhea
d. Susceptibility to respiratory infection

6 Which of the following should the nurse expect to note as a frequent complication for a child with acyanotic congenital heart disease? تكرر في 3 مصادر

كويز 2 – 2024 (أطفال نظري)

a. Susceptibility to respiratory infection
b. Regular bleeding tendencies
c. Frequent vomiting and diarrhea
d. Seizure disorder and coma

7 Which of the following clinical findings would indicate a toxic dose of digoxin? تكرر في 2 مصادر

كويز 2 – 2024 (أطفال نظري)

a. Tachycardia and dysrhythmia
b. Headache and diarrhea
c. Bradycardia and nausea
d. Tinnitus and muscular rigidity

8 Which of the following is most important to be checked before digoxin administration for a 10-month-old child?

كويز 2 شعبة 1 (أطفال نظري)

a. Blood pressure
b. Respiratory rate
c. Temperature
d. Apical pulse

9 Which of the following is a cyanotic congenital heart defect? تكرر في 2 مصادر

كويز 2 شعبة 1 (أطفال نظري)

a. Atrial septal defect (ASD)
b. Ventricular septal defect (VSD)
c. Tetralogy of Fallot (TOF)
d. Patent ductus arteriosus (PDA)

10 Which of the following symptoms should the nurse observe in a newborn with cyanotic CHD?

كويز 2 شعبة 1 (أطفال نظري)

a. Dyspnea, cyanosis, and poor feeding
b. Tachycardia, bounding pulses, and edema
c. Wheezing, cough, and rales on auscultation
d. Cold extremities, bradycardia, and normal skin color

11 The child who had tetralogy of Fallot, a form of congenital heart disease that includes four defects. One of them is:

كويز 2 شعبة 3 (أطفال نظري)

a. Ventricular septal defect
b. Atrial septal defect
c. Pulmonary atresia
d. Aortic stenosis

12 One of the following nursing interventions is most appropriate before digoxin administration for an 18-month-old child: تكرر في 2 مصادر

كويز 2 شعبة 3 (أطفال نظري)

a. Blood pressure
b. Respiratory rate
c. Temperature
d. Apical pulse

13 Which of the following is a cyanotic congenital heart defect? تكرر في 2 مصادر

كويز 2 أطفال نظري

a. Ventricular septal defect (VSD)
b. Atrial septal defect (ASD)
c. Patent ductus arteriosus (PDA)
d. Tetralogy of Fallot (TOF)

14 A child presents with cyanosis and dyspnea during feeding or crying. Which nursing intervention is most appropriate for this "cyanotic spell"?

كويز 2 أطفال نظري

a. Place the child in a knee-to-chest position
b. Place the child in a supine position
c. Encourage immediate oral feeding
d. Administer intravenous antibiotics

15 Which shunt is recommended for cyanosed neonates with Tetralogy of Fallot?

أسئلة سابقة

a. Blalock-Taussig shunt
b. Glenn procedure
c. Fontan procedure
d. Norwood procedure

16 Which condition is associated with a “bounding pulse”?

أسئلة سابقة

a. Aortic stenosis
b. Persistent ductus arteriosus (PDA)
c. Tetralogy of Fallot (TOF)
d. Coarctation of the aorta

17 What is the primary reason for closing PDA in infants?

أسئلة سابقة

a. To prevent arrhythmias
b. To reduce pulmonary vascular disease risk
c. To improve cyanosis
d. To prevent cardiac hypertrophy

18 Which congenital defect is most often detected antenatally during routine fetal scans?

أسئلة سابقة

a. Ventricular septal defect (VSD)
b. Atrial septal defect (ASD)
c. Hypoplastic left heart syndrome (HLHS)
d. Pulmonary stenosis

19 What is the usual timing of treatment for atrial septal defect (ASD)?

أسئلة سابقة

a. Birth
b. 6 months
c. 3-5 years
d. 10 years

20 Which condition involves parallel circulations in the absence of mixing?

أسئلة سابقة

a. Tetralogy of Fallot
b. Transposition of the great arteries (TGA)
c. Coarctation of the aorta
d. Aortic stenosis

21 What is the common presenting feature of coarctation of the aorta in neonates? أسئلة سابقة

a. Continuous murmur
b. Cyanosis
c. Circulatory collapse after ductal closure
d. Bounding pulses

22 Which medication is critical in managing Cyanotic congenital heart defects? أسئلة سابقة

a. Furosemide
b. Digoxin
c. Prostaglandin E1
d. Adenosine

23 What is a key auscultatory finding in pulmonary stenosis? أسئلة سابقة

a. Continuous murmur at the back
b. Ejection systolic murmur with a click
c. Diastolic murmur at the apex
d. Holosystolic murmur

24 What is the definitive management for Tetralogy of Fallot? أسئلة سابقة

a. Medical treatment only
b. Cardiac catheterization
c. Surgical correction
d. Balloon valvuloplasty

25 Which of the following is a cyanotic heart defect? أسئلة سابقة

a. Ventricular septal defect (VSD)
b. Tetralogy of Fallot (TOF)
c. Atrial septal defect (ASD)
d. Persistent ductus arteriosus (PDA)

26 What does a "blue baby" most likely indicate? أسئلة سابقة

a. Acyanotic heart disease
b. Cyanotic heart disease
c. Pulmonary hypertension
d. Left-to-right shunt

27 Which condition involves a left-to-right shunt? أسئلة سابقة

a. Tetralogy of Fallot
b. Transposition of the great arteries
c. Atrial septal defect
d. Hypoplastic left heart syndrome

28 Which defect commonly leads to Eisenmenger syndrome if untreated? أسئلة سابقة

a. Pulmonary stenosis
b. Ventricular septal defect (VSD)
c. Aortic stenosis
d. Coarctation of the aorta

29 Which feature is typically absent in adult coarctation of the aorta? أسئلة سابقة

a. Systemic hypertension in the arms
b. Radio-femoral delay
c. Cyanosis at birth
d. Collateral vessel murmurs

30 Which diagnostic tool is essential for detecting congenital heart disease? أسئلة سابقة

a. ECG
b. Echocardiography
c. MRI
d. Chest X-ray

31 What is a characteristic feature of hypoplastic left heart syndrome? أسئلة سابقة

a. Normal left ventricle size
b. Small left-sided structures
c. Overriding aorta
d. Large ventricular septal defect

32 What is the primary concern in managing transposition of the great arteries? أسئلة سابقة

a. Preventing cyanosis
b. Maintaining ductal patency
c. Avoiding arrhythmias
d. Reducing pulmonary pressure

33 What is the most appropriate initial step for a cyanosed neonate? أسئلة سابقة

a. Start antibiotics
b. Administer prostaglandin E1
c. Perform balloon atrial septostomy
d. Conduct echocardiography

34 What is the primary cause of hypercyanotic spells in TOF? أسئلة سابقة

a. Right ventricular outflow tract obstruction
b. Left-to-right shunting
c. Ductal closure
d. Pulmonary hypertension

35 Which is the preferred treatment for pulmonary stenosis with a pressure gradient >64 mmHg? أسئلة سابقة

a. Surgical repair
b. Balloon dilatation
c. Diuretic therapy
d. Stent placement

36 Which congenital defect is commonly associated with Down syndrome? أسئلة سابقة

a. Tetralogy of Fallot
b. Atrioventricular septal defect (AVSD)
c. Transposition of the great arteries
d. Pulmonary stenosis

37 One of the following nursing interventions is most appropriate before digoxin administration is to check for 18 months old child: تكرر في 2 مصادر أسئلة سابقة

a. Blood pressure.
b. Respiratory rate.
c. Temperature.
d. Apical pulse.

38 Patent ductus arteriosus is an opening between the pulmonary artery and the aorta. (True / False) أسئلة سابقة

a. True
b. False

39 A child with tetralogy of Fallot (a congenital heart disease that includes four defects). One of them is: أسئلة سابقة

a. Ventricular septal defect
b. Atrial septal defect
c. Pulmonary atresia
d. Aortic stenosis

40 The most important nursing intervention for a child with congenital heart disease and on digoxin is: تكرر في 2 مصادر

أسئلة سابقة

a. Assess heart rate.
b. Assess respiratory rate.
c. Assess K level in blood.
d. Assess intake and output.

41 Before giving Digoxin to an infant, the heart rate should not be less than:

أسئلة سابقة

a. 100 beat/min
b. 60 beat / min.
c. 80 beat / min.
d. 120 beat / min.

42 Which of the following should the nurse expect to note as a frequent complication for a child with a cyanotic congenital heart disease? تكرر في 3 مصادر

أسئلة سابقة

a. Susceptibility to respiratory infection
b. Regular bleeding tendencies
c. Frequent vomiting and diarrhea
d. Seizure disorder and coma

43 Which of the following clinical findings would indicate a toxic dose of digoxin? تكرر في 2 مصادر

أسئلة سابقة

a. Tachycardia and dysrhythmia
b. Headache and diarrhea
c. Bradycardia and nausea
d. Tinnitus and muscular rigidity

44 The nurse is to administer Digoxin Elixir to a 6-month-old with a congenital heart defect. The nurse auscultates an apical pulse rate of 100. the nurse should:

أسئلة سابقة

a. Record the heart rate and call the physician
b. Record the heart rate and administer the medication
c. Administer the medication and recheck the heart rate in 30 minutes
d. Hold the medication and recheck the heart rate in 30 minutes.

45 Which of the following can indicate left-sided heart failure in an infant?

أسئلة سابقة

a. Fever
b. Low appetite
c. Increased respiratory rate
d. Crying

46 Congenital anomaly is a structural defect present at birth?

أسئلة سابقة

a. Ture
b. False

47 Cyanotic CHD is:

أسئلة سابقة

a. PDA
b. VSD
c. TGA
d. ASD

48 A 5-year-old with congestive heart failure has been receiving Digoxin (Lanoxin). Which finding indicated that the medication is having a desired effect.

أسئلة سابقة

a. Increased urinary output
b. Stabilized weight
c. Improved appetite
d. Increased pedal edema

49 A nurse is caring for an infant recently diagnosed with a congenital heart defect. Which clinical sign would most likely be present? أسئلة سابقة

a. Slow pulse rate
b. Weight gain
c. Decreased systolic pressure
d. Irregular WBC lab values

50 Foramen oval An opening between two atrium? أسئلة سابقة

a. True
b. False

51 Signs of congestive heart failure is : أسئلة سابقة

a. Edema
b. Cyanosis
c. Anorexia
d. All of Above

52 Condition in which heart cannot pump adequate blood to tissues: أسئلة سابقة

a. Rheumatic fever
b. Cardiomyopathy
c. Pericarditis
d. Congestive heart failure

53 Persistence of a fetal duct between the pulmonary artery and the aorta. أسئلة سابقة

a. Patent dictus arteriosus
b. Circulation of aorta
c. Ventricular septal defect
d. Atrial septal defect

54 The earliest sign of congestive heart failure on a chest X-ray is أسئلة سابقة

a. Increased heart size
b. Kerley B lines
c. Central pulmonary vascular
d. Pleural effusion

55 Management of TGA including: أسئلة سابقة

a. Prostaglandin E1
b. Indomethacin
c. Ibuprofen
d. All of Above

56 Clinical manifestation of cyanotic CHD: أسئلة سابقة

a. Anoxic spells
b. Systolic murmur
c. Diaphoresis
d. Tachycardia

57 An infant with a marked cyanotic congenital heart defect with decreased pulmonary vascularity should be treated with: أسئلة سابقة

a. Digoxin
b. Indomethacin
c. Prostaglandin E1
d. Epinephrine

58 Corrective surgery in management of tetralogy of Fallot done by 1 year? أسئلة سابقة

a. True
b. False

59 Therapy of blue spell in Fallot tetralogy includes all the following, Except أسئلة سابقة

a. Knee chest position
b. Oxygen
c. Morphine
d. Dopamine

60 What is the most common complication of Infective endocarditis? أسئلة سابقة

a. Hematuria.
b. Pericardial effusion.
c. Splenomegaly.
d. Cerebral stroke
e. CHF

61 All of the following are risk factors for developing CHD, EXCEPT: أسئلة سابقة

a. Diabetes mellitus and rubella
b. Alcohol consumption
c. Teratogenic drugs such as anticonvulsants
d. Younger maternal age
e. Secondhand tobacco smoke
f. Increased maternal age

62 Cyanotic congenital heart defect is seen in TGA with increased pulmonary blood flow. (True / False) أسئلة سابقة

a. True
b. False

63 Congenital heart defect may be a component of down syndrome أسئلة سابقة

a. True
b. False

64 Acyanotic congenital heart defects are seen in all of the following, EXCEPT: أسئلة سابقة

a. Aortic stenosis
b. Pulmonic stenosis
c. Tetralogy of Fallot
d. Atrial septal defect

65 ASD is an abnormal opening between the atria allowing blood to flow from the right atrium to the left atrium. (True / False) أسئلة سابقة

a. True
b. False

66 Patent Ductus Arteriosus (PDA) is أسئلة سابقة

a. Failure of the fetal ductus arteriosus to close within the first week of life.
b. Results in right-to-left shift of blood flow from pulmonary artery to the aorta.
c. Results in left-to-right shift of blood flow from aorta to the pulmonary artery.
d. A and B
e. A and C

67 Ventricular Septal Defect (VSD) is أسئلة سابقة

a. Abnormal opening between the Rt. And Lt.Ventricles.
b. Either membranous (20%) or muscular (80%).
c. More than 75% of the cases may close spontaneously.
d. All of the above

68 All of the following are clinical manifestations of acyanotic congenital heart disease EXCEPT: أسئلة سابقة

a. Dyspnea
b. Diaphoresis
c. Bradypnea
d. Systolic murmur

69 For mild ASD the appropriate management is surgical closure at 4–5 years. (True / False) أسئلة سابقة

a. True
b. False

70 Pulmonary blood flow is increased by placing a band around the pulmonary artery as palliative management for VSD. (True / False) أسئلة سابقة

a. True
b. False

71 Complete surgical repair for VSD depends on the presence of pulmonary hypertension and can be performed from one year to school age أسئلة سابقة

a. True
b. False

72 For a premature infant of PDA the medical treatment can be all of the following except: أسئلة سابقة

a. Indomethacin
b. Ibuprofen
c. Prostaglandin E1
d. A and C

73 Surgical correction of PDA is done by ligation via right thoracotomy with cardiopulmonary bypass. (True / False) أسئلة سابقة

a. True
b. False

74 Transportation of great arteries (TGA) is: أسئلة سابقة

a. Abnormal opening between the Rt. And Lt Ventricles.
b. Failure of the fetal ductus arteriosus to close
c. Abnormal opening between the atria allow blood to flow from left atrium to right atrium.
d. Aorta originating from the right ventricle, and pulmonary originating from the left ventricle.

75 As a management for TGA, all of the following can be performed except: أسئلة سابقة

a. Ballon Atrial septostomy
b. Arterial switch procedure
c. Left thoracicotomy
d. Prostaglandin E1 infusion

76 In TGA it is essential to keep the PDA closed, by administering prostaglandin E1 infusion. (True / False) أسئلة سابقة

a. True
b. False

77 Tetralogy Of Fallot (TOF) is the presence of four main defects, choose the wrong one: أسئلة سابقة

a. Pulmonary stenosis
b. Overriding aorta
c. Atrial septal defect (ASD)
d. Right ventricle hypertrophy

78 An infant presented with all the following manifestations: metabolic acidosis, poor growth, exercise intolerance, clubbing fingers, polycythemia and anoxic spells, the nurse expect the infant is experiencing: أسئلة سابقة

a. Atrial septal defect (ASD)
b. Ventricular septal defect (VSD)
c. A Cyanotic CHD
d. Cyanotic CHD

79 Restless attacks may follow feeding or defecation as a result of low O2 levels in the blood cause the lips, fingers, and toes to look blue, and lead to unconsciousness, convulsion, and death is which of the following: أسئلة سابقة

a. Clubbing fingers
b. Polycythemia
c. Anoxic spells
d. None of the above

80 For treating hypercyanotic spells, all of the following can be done EXCEPT: أسئلة سابقة

a. Place infant in knee-chest (squatting) position
b. Employ a calm, comforting approach
c. Administer 100% oxygen by face mask
d. Give morphine subcutaneously or IV
e. Place infant in prone position
f. Begin IV fluid replacement and volume expansion if needed

81 Management of Tetralogy of Fallot are which of the following أسئلة سابقة

a. Prophylactic antibiotics
b. Palliative shunt
c. Corrective surgery
d. All of the above

82 Which of the following can be a long-term complication of TOF? أسئلة سابقة

a. Cerebral thrombosis
b. Viral endocarditis
c. Urinary tract infection
d. Renal failure

83 The aim of performing a palliative shunt in TOF (Tetralogy of Fallot) is to أسئلة سابقة

a. Increase aortic artery blood flow.
b. Decrease pulmonary artery blood flow.
c. Increase pulmonary artery blood flow.
d. Decrease aortic artery blood flow.

84 Is the inability of the heart to pump an adequate amount of blood to the systemic circulation to meet the metabolic demands of the body. أسئلة سابقة

a. ASD
b. TGA
c. TOF
d. CHF

85 In Right-sided heart failure the following are seen except: أسئلة سابقة

a. Increased pressure in right atrium
b. Decreased systemic venous circulation
c. Hepato-splenomegaly
d. Edema

86 In Left-sided heart failure which of the following can be seen: أسئلة سابقة

a. Increased Pressure in left atrium
b. Increased pressure in pulmonary veins
c. Pulmonary edema
d. All of the above

87 The inability of the heart to maintain an adequate cardiac output to kidneys (myocardial failure) lead to: أسئلة سابقة

a. Increased sodium and water reabsorption
b. Fluid overload (increase workload on heart).
c. Congestion in pulmonary and systemic circulation
d. All of the above

88 An infant presented with Tachypnea, Wheezing, Crackles, Retractions, Dyspnea, Orthopnea cough, grunting, flaring nares, irritability, tiring with feeding, hoarseness, and cyanosis. The nurse expected that the infant developed أسئلة سابقة

a. Systemic Venous Congestion
b. Atrial septal defect
c. Ventricular septal defect
d. Pulmonary Venous Congestion

89 Giving the following manifestations for a child: Hepatomegaly, Ascites, Peripheral Edema and Neck Vein Distention, the nurse can expect that the child is experiencing which of the following: أسئلة سابقة

a. Ventricular septal defect
b. Pulmonary Venous Congestion
c. Systemic Venous Congestion
d. Atrial septal defect

90 Which of the following can be a clinical manifestation for Impaired myocardial function: أسئلة سابقة

a. Bradycardia and Sweating.
b. Weak Peripheral Pulses and hypertension.
c. Capillary refill time less than 2 seconds.
d. Decreased urine output (oliguria).

91 Therapy of CHF includes the following EXCEPT: أسئلة سابقة

a. Digoxin
b. Captopril (Capoten)
c. O2 therapy
d. Propranolol
e. Furosemide (Lasix)

92 Mr. and Mrs. Baker's only daughter is diagnosed with heart failure. Which of the following interventions would be appropriate to promote optimal nutrition for the infant? أسئلة سابقة

a. Replacing regular nipples with easy-to-suck ones
b. Allowing the infant to feed for at least 1 hour
c. Providing large feedings evenly spaced every 4 hours
d. Offering formula that is high in sodium and calories

93 The main action of Digoxin is all the following except: أسئلة سابقة

a. Increase cardiac contractility
b. Increase cardiac output
c. Increase heart rate
d. A and C

94 The following can be side effects of digoxin EXCEPT: أسئلة سابقة

a. Dysrhythmias
b. Anorexia
c. Visual disturbances
d. Tachycardia
e. Nausea and vomiting

95 Before administration of Digoxin the nurse should do which of the following: أسئلة سابقة

a. Take the apical pulse for 1 minute.
b. Give digoxin at regular intervals.
c. Administer the drugs carefully by slowly.
d. Not mix it with other foods or fluids.
e. All of the above.

96 For a child who receives Digoxin, he should be given water after administering the drug or brush the teeth أسئلة سابقة

a. True
b. False

97 Which of the following is a cyanotic congenital heart disease? أسئلة سابقة

a. Atrial septal defect (ASD)
b. Ventricular septal defect (VSD)
c. Tetralogy of Fallot (TOF)
d. Patent ductus arteriosus (PDA)

98 What is the most common congenital heart defect? أسئلة سابقة

a. Coarctation of the aorta (CoA)
b. Atrial septal defect (ASD)
c. Ventricular septal defect (VSD)
d. Transposition of the great arteries (TGA)

99 What is the key pathophysiological feature of right-to-left shunt defects? أسئلة سابقة

a. Pulmonary congestion
b. Decreased systemic oxygenation
c. Increased pulmonary blood flow
d. Hypertrophy of the left ventricle

100 Which diagnostic test is considered the gold standard for detecting congenital heart disease? أسئلة سابقة

a. Electrocardiogram (ECG)
b. Chest X-ray
c. Echocardiography (ECHO)
d. Pulse oximetry

101 Which maternal condition increases the risk of congenital heart disease in the fetus? أسئلة سابقة

a. Hypothyroidism
b. Hyperlipidemia
c. Diabetes
d. Migraine

102 What structural defect is present in transposition of the great arteries (TGA)? أسئلة سابقة

a. A single artery replaces the aorta and pulmonary artery
b. The aorta and pulmonary artery are switched
c. A missing tricuspid valve
d. A narrowed aortic valve

103 What is a key clinical feature of coarctation of the aorta (CoA)? أسئلة سابقة

a. Bounding pulses in the lower extremities
b. Murmur heard best at the apex
c. Weak femoral pulses and hypertension in upper limbs
d. Cyanosis in all extremities

104 Which treatment keeps the ductus arteriosus open in ductal-dependent heart defects? أسئلة سابقة

a. Diuretics
b. ACE inhibitors
c. Prostaglandin E1
d. Beta-blockers

105 What is the primary reason for polycythemia in cyanotic congenital heart disease? أسئلة سابقة

a. Increased pulmonary congestion
b. Chronic hypoxia leading to increased erythropoiesis
c. Reduced cardiac output
d. Left ventricular hypertrophy

106 Which preventive measure helps reduce the risk of congenital heart disease? أسئلة سابقة

a. High-protein diet during pregnancy
b. Early childhood vaccinations
c. Avoiding teratogenic medications
d. Daily aspirin during pregnancy

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

N1 Which defect produces a left-to-right shunt and is acyanotic? NCLEX

a. Tetralogy of Fallot
b. Transposition of great arteries
c. Ventricular septal defect
d. Tricuspid atresia

N2 The four defects of tetralogy of Fallot are: NCLEX

a. ASD, VSD, PDA, coarctation
b. Pulmonary stenosis, VSD, overriding aorta, right ventricular hypertrophy
c. Aortic stenosis, ASD, LVH, PDA
d. Mitral stenosis, VSD, ASD, PDA

N3 Select ALL nursing actions for a hypercyanotic (Tet) spell. SELECT ALL

a. Knee-chest position
b. Calm, comforting approach
c. 100% oxygen
d. Morphine as prescribed
e. Place supine flat and leave alone

N4 Before giving digoxin to an infant, the nurse counts the apical pulse for: NCLEX

a. 15 seconds
b. 30 seconds
c. 1 full minute
d. 2 minutes

N5 **Which drug keeps the ductus arteriosus open in ductal-dependent lesions such as TGA?** NCLEX

a. Indomethacin
b. Ibuprofen
c. Prostaglandin E1
d. Digoxin

N6 **An infant with CHF has poor feeding. Which intervention promotes nutrition?** NCLEX

a. Large feedings every 4 hours
b. Small frequent feedings with soft easy-to-suck nipples
c. Feed for at least 1 hour
d. High-sodium formula

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Acyanotic (L→R)	VSD, ASD, PDA — increased pulmonary blood flow.	**Obstructive**	Coarctation, aortic stenosis, pulmonary stenosis.
Cyanotic (R→L)	TOF, TGA, tricuspid atresia.	**Most common CHD**	VSD.
TOF	PS + VSD + overriding aorta + RVH.	**Tet spell care**	Knee-chest, calm, 100% O2, morphine, IV fluids.
TGA	Aorta from RV, pulmonary artery from LV; needs mixing — PGE1, balloon septostomy, arterial switch.	**PDA**	Duct between aorta and pulmonary artery; bounding pulse, machinery murmur; indomethacin/ibuprofen in preterm.
Coarctation	Narrowed aorta; high BP arms, weak femoral pulses, radio-femoral delay.	**Digoxin**	Count apical pulse 1 min; toxicity: bradycardia, nausea, vomiting, visual changes.
CHF in infant	Tachypnea, tiring with feeds, sweating, hepatomegaly, weight gain.	**Gold standard diagnosis**	Echocardiography.

Answer Key

الإجابات

1. d — Give dose if heart rate is 100 beats/min

2. a — Blood shunt from left to right

3. a — True

4. a — Assess heart rate

5. d — Susceptibility to respiratory infection

6. a — Susceptibility to respiratory infection

7. c — Bradycardia and nausea

8. d — Apical pulse

9. c — Tetralogy of Fallot (TOF)

10. a — Dyspnea, cyanosis, and poor feeding
الكويز ده مفيهوش إجابة معلّمة — إجابة مقترحة.

11. a — Ventricular septal defect

12. d — Apical pulse

13. d — Tetralogy of Fallot (TOF)

14. a — Place the child in a knee-to-chest position

15. a — Blalock-Taussig shunt

16. b — Persistent ductus arteriosus (PDA)

17. b — To reduce pulmonary vascular disease risk

18. c — Hypoplastic left heart syndrome (HLHS)

19. c — 3-5 years

20. b — Transposition of the great arteries (TGA)

21. c — Circulatory collapse after ductal closure

22. c — Prostaglandin E1

23. b — Ejection systolic murmur with a click

24. c — Surgical correction

25. b — Tetralogy of Fallot (TOF)

26. b — Cyanotic heart disease

27. c — Atrial septal defect

28. b — Ventricular septal defect (VSD)

29. c — Cyanosis at birth

30. b — Echocardiography

31. b — Small left-sided structures

32. b — Maintaining ductal patency

33. b — Administer prostaglandin E1

34. a — Right ventricular outflow tract obstruction

35. a — Surgical repair
الملف معلّم Surgical repair، لكن العلاج المفضّل للـ PS مع gradient > 64 mmHg هو balloon dilatation (valvuloplasty). الجراحة للحالات اللي البالون فشل فيها.

36. b — Atrioventricular septal defect (AVSD)

37. d — Apical pulse.

38. a — True

39. a — Ventricular septal defect

40. a — Assess heart rate.

41. a — 100 beat/min
نفس السؤال ظهر بـ 0 من (ملف pediatric questions) في صورة امتحان 1 مع اختيار 100 — المراجع بتقول امنع الجرعة لو النبض > 90–110 في الرضيع، فاعتمد 100 زي بنك الدفعة.

42. a — Susceptibility to respiratory infection

43. c — Bradycardia and nausea

44. b — Record the heart rate and administer the medication

45. c — Increased respiratory rate

46. a — Ture

47. c — TGA

48. a — Increased urinary output

49. b — Weight gain

50. a — True

51. d — All of Above

52. d — Congestive heart failure

53. a — Patent dictus arteriosus

54. a — Increased heart size

55. a — Prostaglandin E1

56. a — Anoxic spells

57. c — Prostaglandin E1

58. a — True

59. d — Dopamine

60. c — CHF

61. d — Younger maternal age

62. b — False

63. a — True

64. c — Tetralogy of Fallot

65. b — False

66. e — A and C

67. a — Abnormal opening between the Rt. And Lt.Ventricles.

68. c — Bradypnea

69. b — False

70. b — False

71. a — True

72. c — Prostaglandin E1

73. b — False

74. d — Aorta originating from the right ventricle, and pulmonary originating from the left ventricle.

75. c — Left thoracicotomy

76. b — False

77. c — Atrial septal defect (ASD)

78. d — Cyanotic CHD

79. c — Anoxic spells

80. e — Place infant in prone position

81. d — All of the above

82. a — Cerebral thrombosis

83. c — Increase pulmonary artery blood flow.

84. d — CHF

85. b — Decreased systemic venous circulation

86. d — All of the above

87. d — All of the above

88. d — Pulmonary Venous Congestion

89. c — Systemic Venous Congestion

90. d — Decreased urine output (oliguria).

91. d — Propranolol
Beta-blockers (propranolol) مش من علاج الـ CHF الحاد في الأطفال حسب البنك.

92. a — Replacing regular nipples with easy-to-suck ones

93. c — Increase heart rate

94. d — Tachycardia

95. e — All of the above.

96. a — True

97. c — Tetralogy of Fallot (TOF)
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

98. c — Ventricular septal defect (VSD)
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

99. b — Decreased systemic oxygenation
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

100. c — Echocardiography (ECHO)
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

101. c — Diabetes
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

102. b — The aorta and pulmonary artery are switched
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

103. c — Weak femoral pulses and hypertension in upper limbs
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

104. c — Prostaglandin E1
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

105. b — Chronic hypoxia leading to increased erythropoiesis
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

106. c — Avoiding teratogenic medications
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

N1. c — Ventricular septal defect
VSD, ASD and PDA shunt blood left to right — acyanotic, with increased pulmonary flow.

N2. b — Pulmonary stenosis, VSD, overriding aorta, right ventricular hypertrophy
TOF = PS + VSD + overriding aorta + RVH.

N3. a, b, c, d — Knee-chest position; Calm, comforting approach; 100% oxygen; Morphine as prescribed
Knee-chest increases systemic resistance; oxygen and morphine reduce hypoxia and infundibular spasm.

N4. c — 1 full minute
Count apical pulse for one full minute; hold if below the ordered limit (infant ~90–110).

N5. c — Prostaglandin E1
PGE1 maintains ductal patency; indomethacin/ibuprofen close it.

N6. b — Small frequent feedings with soft easy-to-suck nipples
Small frequent feeds with soft nipples reduce energy expenditure; limit feeding time to about 30 minutes.